

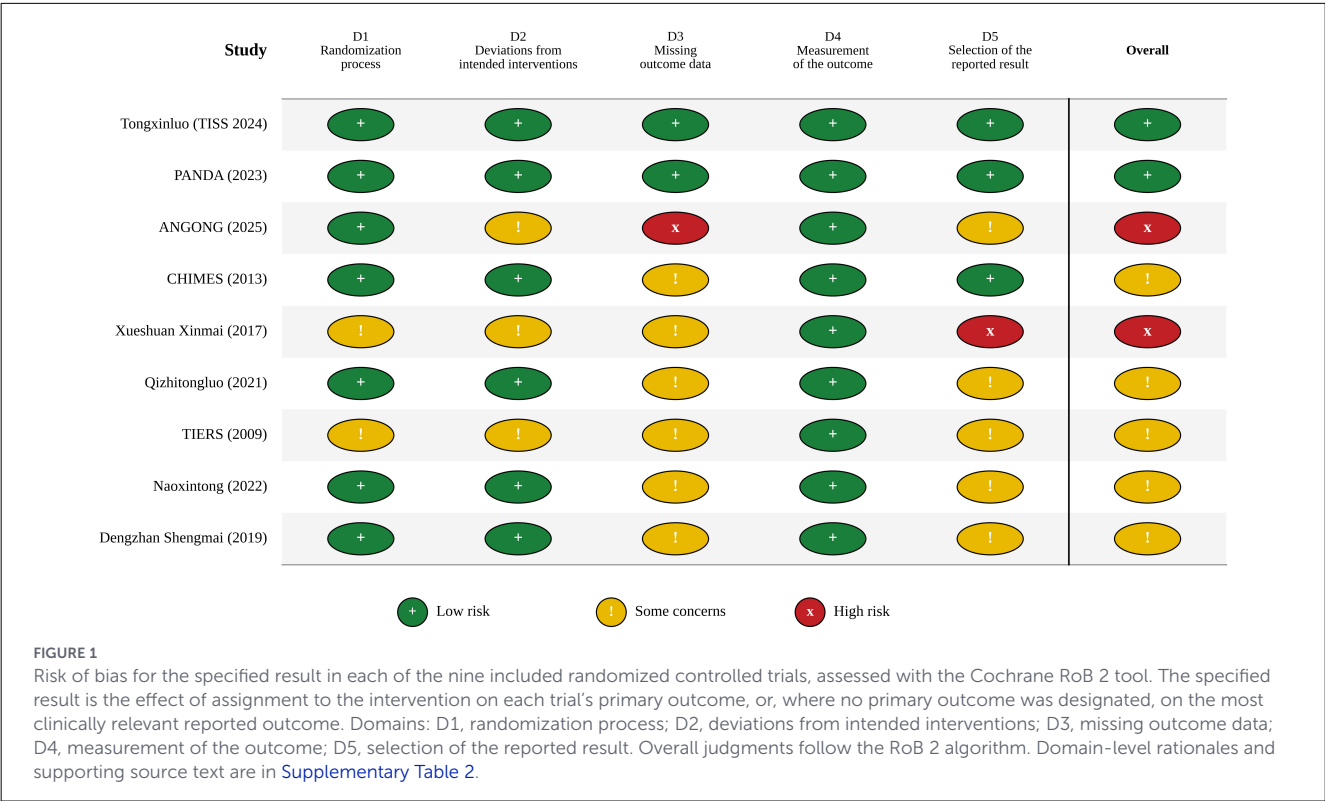


TABLE 1 RoB 2 judgments for the specified result in each included trial.

Trial (specified result)	D1 randomization	D2 deviations	D3 missing data	D4 measurement	D5 selective reporting	Overall
Tongxinluo (mRS 0-1 at 90 d)	Low	Low	Low	Low	Low	Low
PANDA (mRS 0-2 at 3 mo)	Low	Low	Low	Low	Low	Low
ANGONG (infarct and edema volume at 14 d)	Low	Some concerns	High	Low	Some concerns	High
CHIMES (mRS shift at 3 mo)	Low	Low	Some concerns	Low	Low	Some concerns
Xueshuan Xinmai (NIHSS at 2 wk)	Some concerns	Some concerns	Some concerns	Low	High	High
Qizhitongluo (FMMS-LL at 12 wk)	Low	Low	Some concerns	Low	Some concerns	Some concerns
TIERS (Fugl-Meyer at 4 and 8 wk)	Some concerns	Some concerns	Some concerns	Low	Some concerns	Some concerns
Naoxintong (recurrence at 2 y)	Low	Low	Some concerns	Low	Some concerns	Some concerns
Dengzhan Shengmai (stroke at 1 y)	Low	Low	Some concerns	Low	Some concerns	Some concerns

Assessments relate to the effect of assignment to the intervention on each trial's primary outcome, or, for Xueshuan Xinmai, on the most clinically relevant reported outcome, since that report designates no primary outcome. Overall judgments follow the RoB 2 algorithm and are never more favorable than the least favorable contributing domain. Domain rationales with supporting source text are in [Supplementary Table 2](#). This table, [Figure 1](#) and the GRADE profiles are generated from a single master dataset.

at different doses and routes, started between 36 h and 6 months after onset, assessed with different primary outcome instruments over 14 days to 24 months. Two trials reporting a binary functional outcome at approximately 3 months do so at different thresholds, mRS 0 to 1 and mRS 0 to 2, which are related but not interchangeable. We note explicitly that seven of the nine trials

report a primary outcome with a point estimate and confidence interval, so data availability was not the constraint. Synthesis followed SWiM guidance (17). Trials were grouped first by clinical phase, defined by the interval between onset and start of the study intervention together with the trial's stated aim, then by formulation. For each trial we report the analysis